

Office of Enforcement and Removal Operations  
ICE Health Service Corps

U.S. Department of Homeland Security  
500 12th Street, SW  
Washington, D.C. 20536



U.S. Immigration  
and Customs  
Enforcement

October 27, 2020

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE  
Assistant Director  
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

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CAPT (b)(6),(b)(7)(C),(b)(7)(F) RN  
Chief, Investigations Unit  
ICE Health Service Corps

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FROM:

CDR (b)(6),(b)(7)(C),(b)(7)(F) PA-C  
Senior Investigator  
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings  
Orlan Ariel CARCAMO Navarro, (b)(6),(b)(7)(C)

**Executive Summary:**

On March 18, 2020, U.S. Immigrations and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Orlan Ariel CARCAMO Navarro. Mr. CARCAMO, a 27-year-old Honduran male, was in ICE custody from February 21 to March 18, 2020, and assigned to Karnes County Residential Center (KCRC) in Karnes City, Texas (TX), on the date of his death. An autopsy ruled Mr. CARCAMO's cause of death as a suicide due to hanging.

On February 19, 2020, Mr. CARCAMO, accompanied with his (b)(6),(b)(7)(C) (b)(6),(b)(7)(C) at the U.S. Customs and Border

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Protection's (USBP) Presidio, TX port of entry. CBP served Mr. CARCAMO a Notice and Order of Expedited Removal, Form I-860, charging removability pursuant to the Immigration and Nationality Act, as an immigrant without a valid, unexpired immigrant visa or other document required for admission.

On February 21, 2020, CBP transferred Mr. CARCAMO to KCRC, pending an interview with a U.S. Citizenship and Immigration Services (USCIS) (b)(6),(b)(7)(C).

On February 28, 2020, (b)(6),(b)(7)(C)

(b)(6),(b)(7)(C)

On March 11, 2020, (b)(6),(b)(7)(C) and returned the case to ICE for removal.

On March 18, 2020 at approximately 12:30 a.m., a KCRC's residential advisor (RA) conducted a routine security check, discovered Mr. CARCAMO hanging from the shower rod in his room, and immediately called for emergency medical assistance.

At approximately 12:32 a.m., KCRC's medical and security staff responded to the scene, initiated cardio-pulmonary resuscitation (CPR), and called for emergency medical services (EMS) via 911.

At approximately 12:42 a.m., Karnes County EMS (KCEMS) arrived on scene, noted Mr. CARCAMO's body was cold to touch, his mouth was difficult to open, and he was in cardiac arrest. KCEMS intubated Mr. CARCAMO, started intraosseous fluids (a process used to inject medication and fluids directly into the bone marrow), and advanced life-support measures.

At approximately 12:55 a.m., KCEMS pronounced Mr. CARCAMO deceased, after failed resuscitation attempts.

### **Mortality Finding:**

As a result of Mr. CARCAMO's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) determined that Mr. CARCAMO's medical care at KCRC was not provided within the safe limits of practice. Specifically, the MRC determined, care or lack thereof, could pose a future risk. Overall findings identified were: 1) untimely mental health

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follow-up appointments; 2) lack of continuity of care; 3) missing and/or lack of documentation; 4) delay in KCEMS entering the facility the day of Mr. CARCAMO's death; and 5) nursing staff did not deliver the correct compression to breath ratio of 30:2 during each cycle of CPR.

The MRC convened on August 4, 2020, and consisted of the following members: (b)(6),(b)(7)(C),(b)(7)(F) (b)(6),(b)(7)(C),(b)(7)(F) Doctor of Osteopathic Medicine (DO), Houston Detention Center's Clinical Director, physician reviewer; Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) registered nurse (RN), Western Regional Nurse Manager, nurse reviewer; CDR (b)(6),(b)(7)(C),(b)(7)(F) physician assistant, advanced practice provider (APP) reviewer; and CDR (b)(6),(b)(7)(C),(b)(7)(F) Behavioral Health Consultant, mental health reviewer.

The following summary of events was based on: 1) Mr. CARCAMO's KCRC medical records and EMS report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. CARCAMO's KCRC detention file; 5) Mr. CARCAMO's death certificate and autopsy report; 6) video footage from March 18, 2020; 7) a remote site review and staff interviews conducted by fact-finder CDR (b)(6),(b)(7)(C),(b)(7)(F) and Western Regional Behavioral Health Consultant, LCDR (b)(6),(b)(7)(C),(b)(7)(F) and 9) applicable IHSC directives and ICE Detention Standards.

### *Summary of Events*

**On February 21, 2020**, an RN completed Mr. CARCAMO's intake screening, identified him as a Spanish speaker, documented Mr. CARCAMO's history of a right foot fracture due to an accident (five years ago), and reported threats from drug traffickers. Mr. CARCAMO denied having any pain, mental health issues, hospitalizations, taking psychotropic medications, previous suicide attempts, or suicidal ideations. The RN referred Mr. CARCAMO for further evaluation to a physician, dentist, and mental health provider.

**On February 22, 2020**, an APP completed Mr. CARCAMO's physical assessment, documented normal findings, and instructed Mr. CARCAMO to follow up, as needed.

**On February 24, March 2, and 9, 2020**, a licensed master social worker (LMSW) completed Mr. CARCAMO's weekly parent audit check and documented normal findings.

**On March 5, 2020**, a mental health caseworker (MHCW) completed Mr. CARCAMO's individual trauma session group assessment form, documenting his active participation, neatly groomed appearance, calm mood, appropriate affect, and moderate attentiveness. During the session, Mr. CARCAMO started crying, left the session, stated he did not require mental health services anymore, and walked to the library. The MHCW followed Mr. CARCAMO to the library, and he informed her that he became upset when discussing his experiences and the negative news he received about his case, approximately a week ago. Mr. CARCAMO denied having any suicidal ideations. The MHCW completed Mr. CARCAMO's refusal form for group

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and individual psychiatric services, reminded Mr. CARCAMO about his access to future mental health services, if he changed his mind, and contacted KCRC's chaplain to speak with Mr. CARCAMO.

**On the same day**, the KCRC's chaplain reported to the library; however, Mr. CARCAMO did not speak to her. The chaplain reported Mr. CARCAMO wrote a letter about his experience in his country, his journey, (b)(6),(b)(7)(C), and not understanding the reason for his negative case findings; however, he did not provide her with the letter. The actual date or verbiage included in the letter could not be verified, since the document was not located within his property, nor was it forwarded to ICE.

**On March 6, 2020**, a KCRC's case manager (CM), sent email notification to KCRC's mental health team, requesting Mr. CARCAMO's mental health evaluation, since he received negative results in court and appeared upset and sad.

**On March 10, 2020**, in accordance with the CM's request, Mr. CARCAMO attended a stress management group session. An MHCW documented Mr. CARCAMO's minimal participation, neatly groomed appearance, moderate alertness, and appropriate affect. The MHCW documented Mr. CARCAMO did not require any additional referrals.

**On March 15, 2020**, an APP evaluated Mr. CARCAMO for pain due to a right thigh abscess. The APP ordered an antibiotic and pain-relieving medication.

**On March 16, 2020**, an LMSW completed Mr. CARCAMO's weekly parent audit check, documented his reports of having anxiety, insomnia (difficulty sleeping), and referred Mr. CARCAMO for an individual psychiatric evaluation. The LMSW noted Mr. CARCAMO was alert/attentive, calm, cheerful, but with adjustment problems, and requested an individual mental health evaluation. Mr. CARCAMO did not receive an individual mental health evaluation with a psychologist or psychiatrist prior to his death. KCRC housed Mr. CARCAMO and (b)(6),(b)(7)(C) in the short stay unit (SSU), since (b)(6),(b)(7)(C).

**On March 18, 2020**, KCRC's policies require an RA to perform (b)(7)(E) minute security checks in the SSU, but the assigned RA failed to enter Mr. CARCAMO's room each time to check and confirm his presence of life.

- **At approximately 12:30 a.m.**, during a security check, the RA noted Mr. CARCAMO was not in his bed, and the room lights were off. The RA turned the lights on, opened Mr. CARCAMO's room door, observed Mr. CARCAMO hanging, and announced a medical emergency aloud and over radio transmission.
- **At approximately 12:32 a.m.**, security and nursing staff entered Mr. CARCAMO's room, observed him hanging from a shower rod with a bed sheet, requested a cut down

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tool, and activated EMS via 911. The nursing staff cut the ligature from around Mr. CARCAMO's neck, lowered his body to the floor, initiated CPR, attached an automated external defibrillator (AED) until EMS arrived, but failed to apply a collar device for immobilization, did not initially attach the AED pads onto his chest correctly, and did not consistently perform the appropriate compression to breath ratio of 30:2, during each cycle of CPR.

- **At approximately 12:42 a.m.**, KCEMS arrived on scene, noted Mr. CARCAMO's body was cold to touch, his mouth was difficult to open, and he was in cardiac arrest. KCEMS intubated Mr. CARCAMO, started intraosseous fluids, and advanced life-support measures.
- **At approximately 12:55 a.m.**, KCEMS pronounced Mr. CARCAMO deceased, after failed resuscitation attempts.

ICE detention standards used for this review: ICE Family Residential Standards (FRS) 2007. The following are identified program strengths, weaknesses, and recommendations found during this review.

### ***Strengths***

Strengths identified during this investigation were: 1) an RN's prompt referral to a higher-level care, when underlying medical conditions were identified; 2) timely KCRC medical emergency response; and 3) the CM's prompt notification to the mental health team after Mr. CARCAMO received negative findings at a court case.

### ***Weaknesses***

#### **1. Accreditation.**

- KCRC's health services administrator (HSA) initially reported having a waiver that excluded KCRC from maintaining compliance with the Joint Commission (JC) accreditation standards, in accordance with the ICE FRS 2007. ICE FRS 2007 states, "*The health care program and the medical facilities shall be under the direction of an HSA and shall be accredited and maintain compliance with the standards of the Joint Commission on the Accreditation of Health Care Organizations (JCAHO).*" Review of the waiver did not contain any indication precluding KCRC from maintaining the JC standards.

*Applicable standards of care for this finding:*

- FRS 2007: 4.3, Medical Care, section II., *Expected Practices*.

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## 2. Continuity and coordination of care.

- On March 6, 2020, a CM sent email notification to KCRC's mental health team requesting Mr. CARCAMO receive a mental health evaluation, since he previously received negative results in court and appeared upset and sad; however, he did not receive a mental health evaluation to discuss or address his concerns.
- On March 16, 2020. An MHCW requested Mr. CARCAMO receive a higher-level mental health evaluation with a psychologist. The MHCW entered his name and symptoms into the psychologist's referral logbook; but, there was no psychologist on-site, or available, until March 19, 2020. Mr. CARCAMO did not receive a follow-up evaluation after this encounter, or prior to the day of his death.

*Applicable standards of care for this finding:*

- FRS 2007: 4.3, Medical Care, section II., *Expected Practices*.
- FRS 2007: 4.3, Medical Care, section V.W., *Special Needs and Close Medical Supervision*.

## 3. Patient assessments and documentation.

- KCRC's mental health team encountered Mr. CARCAMO on multiple occasions, but failed to document a thorough assessment and findings during each encounter.
- KCRC's health record format did not include all essential elements of a health record. For example, failure of: 1) identifying information on every page, 2) clear and concise progress notes addressing significant findings, diagnoses, treatments, and dispositions, 3) clear and concise medical orders, 4) clear and concise clinical encounters, and 5) signature/titles of each person documenting notes and/or orders into the medical record.
- On March 5, 2020, an MHCW completed Mr. CARCAMO's individual trauma session and group assessment form, documenting his active participation, neatly groomed appearance, calm mood, appropriate affect, and moderate attentiveness; however, Mr. CARCAMO did not participate in a group trauma session, and the MHCW failed to document a thorough assessment and findings.
- On March 18, 2020, during the medical emergency, Mr. CARCAMO's son woke up and potentially witnessed Mr. CARCAMO's hanging. The medical records did not include any documentation to confirm whether his (b)(6),(b)(7)(C)  
(b)(6),(b)(7)(C)

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- On March 14, 2020, an RN evaluated Mr. CARCAMO for pain, due to a skin infection. The RN contacted the on-call MD, received a verbal order to start medications, and stamped the nursing assessment protocol note with the MD's stamp. The MD did not sign or perform a confirmatory sign-off.

*Applicable standards of care for this finding:*

- FRS 2007: 4.3, Medical Care, section II., *Expected Practices*.

#### **4. Medical emergency.**

- On March 18, 2020, during Mr. CARCAMO's medical emergency, medical staff failed to use cervical spine precautions to immobilize Mr. CARCAMO's neck and avoid further injury, failed to appropriately place the AED pads in the appropriate position, and did not perform CPR using the appropriate compression to breath ratio of 30:2 throughout the delivery of all CPR cycles. KCRC's HSA was informed of this issue requiring immediate training to ensure compliance with the 2015 American Heart (AHA) guidelines.
- On March 18, 2020, there was approximately a five-minute delay in opening KCRC's entrance gate for EMS transport to enter the facility.
- On March 18, 2020, Mr. CARCAMO used a sheet as a ligature, wrapped it around a shower rod, and his neck, to hang himself. The shower rod did not collapse or break when Mr. CARCAMO applied his weight to the shower rod.

*Applicable standards of care for this finding:*

- FRS 2007: 4.3, Medical Care, V Expected outcomes, section 14., *First Aid and Medical Emergencies*.
- FRS 2007: 4.3, Medical Care, V Expected outcomes, section 17., *Special Needs and Close Medical Supervision*.
- AHA BLS Health Care Provider Resuscitation and Emergency Cardiovascular Care (ECC) Guidelines (2015), Part 5: Adult Basic Life Support and CPR. Accessed on August 30, 2020. <https://eccguidelines.heart.org/index.php/circulation/cpr-ecc-guidelines-2/part-5-adult-basic-life-and-cpr-quality/>.
- Up to Date, *Evaluation and initial management of cervical spinal column injuries in adults* (2020). Accessed August 30, 2020, at [https://uptodate.com/contents/evaluation-an-initial-management-of-cervical-spinal-column-injuries-in-adults?source=history\\_widget](https://uptodate.com/contents/evaluation-an-initial-management-of-cervical-spinal-column-injuries-in-adults?source=history_widget).

#### **Recommendations**

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- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD of HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, the KSPC administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.